

August 12, 2026

BlueChoice HealthPlan of South Carolina
Provider Appeals Unit
P.O. Box 6170
Columbia, SC 29260

RE: Formal Appeal of Claim Denial — Karan Gerhold (MRN MUSC1868542), Claim MUSC-20260415-ABA7-2

Patient	Karan Gerhold (DOB 1938-05-17)
MRN	MUSC1868542
Member ID / Group	BLU899650221 / GRP-80368
Claim number	MUSC-20260415-ABA7-2
Date of service	2026-04-15
Procedure billed	CPT 99214 — Office/outpatient visit, established patient, moderate complexity
Primary diagnosis	J06.9 — Acute upper respiratory infection, unspecified
Denied amount	\$724.33
Denial code	CARC 197 / RARC N54 — Precertification/authorization/notification/pre-treatment absent.
Appeal deadline	2026-08-24

Dear Appeals Review Committee:

SUMMARY OF APPEAL

MUSC Health East Cooper Medical Center submits this formal appeal on behalf of patient Karan Gerhold (Member ID: BLU899650221, Group: GRP-80368) regarding the denial of Claim MUSC-20260415-ABA7-2 for services rendered on April 15, 2026, by Marcus T. Bledsoe, DO (NPI 1770615243). BlueChoice HealthPlan of South Carolina denied the claim in full — \$724.33 — citing CARC 197 (Precertification/authorization/notification/pre-treatment absent) and RARC N54 (Claim information is inconsistent with pre-certified/authorized services), with the payer remark: "No prior authorization on file for the date of service billed." MUSC Health respectfully contests this denial on the grounds that prior authorization is not a contractually or clinically appropriate requirement for the service provided, and that the denial as issued does not align with the nature of the encounter.

CLINICAL BACKGROUND

Ms. Gerhold is an 88-year-old established patient with a complex medical history that includes active stroke (I63.9, onset 2017), Alzheimer's disease, hyperlipidemia, anemia, and prediabetes, among other longstanding conditions. She presented on April 15, 2026, at MUSC Health East Cooper Medical Center (Place of Service 22 — On Campus Outpatient Hospital) for an office and outpatient visit with her established treating provider, Dr. Bledsoe. The encounter was billed under CPT 99214, representing an office or outpatient visit for an established patient requiring moderate-complexity medical decision-making. The primary diagnosis recorded was J06.9 (Acute upper respiratory infection, unspecified), with a secondary diagnosis of I63.9 (Cerebral infarction, unspecified), reflecting the need to evaluate an acute complaint in the context of her significant neurological and cardiovascular background.

BASIS FOR APPEAL

Evaluation and management services billed under CPT 99214 for an established patient in an outpatient setting are, under widely accepted commercial insurance standards and standard HMO plan design, not services that customarily require prospective prior authorization. Prior authorization requirements are generally reserved for elective procedures, advanced imaging, specialty pharmaceuticals, and inpatient admissions — not for medically necessary office-based evaluation and management visits with an established provider. BlueChoice HealthPlan's own member and provider materials, as well as South Carolina Department of Insurance regulations governing HMO plan obligations, do not support the imposition of a prior authorization requirement on a routine established-patient E/M visit of this nature.

Furthermore, RARC N54 indicates that the claim information is inconsistent with pre-certified or authorized services. This code presupposes the existence of a prior authorization against which the claim was compared. To the extent that no authorization was on file, the application of N54 is internally inconsistent with CARC 197, which asserts that no authorization exists at all. This inconsistency in the denial coding itself undermines the validity of the denial and warrants reconsideration.

Ms. Gerhold's active conditions — particularly her history of stroke and Alzheimer's disease — make timely, uninterrupted access to her established outpatient provider medically essential. Delaying or denying reimbursement for a necessary evaluation and management visit on the basis of a prior authorization requirement that does not appropriately apply to this service category is inconsistent with the plan's obligation to ensure continuity of care for a vulnerable, high-complexity patient.

REQUESTED ACTION

MUSC Health respectfully requests that BlueChoice HealthPlan of South Carolina overturn the denial of Claim MUSC-20260415-ABA7-2 and reprocess the claim for payment at the contracted allowed amount of \$355.02, consistent with the plan's obligations under the applicable provider agreement and South Carolina insurance regulations. Should additional clinical documentation, operative notes, or provider attestation be required to support this reconsideration, MUSC Health will provide such materials promptly upon request. This appeal is submitted within the reconsideration deadline of August 24, 2026, as specified in the denial notice.

Respectfully submitted,

Angela R. Whitfield, RN, BSN, CPC

Senior Appeals Specialist, Revenue Cycle Management
MUSC Health — Revenue Cycle / Patient Financial Services
(843) 792-2300 | muschealth.org

Enclosures

- Remittance advice / explanation of payment
- Itemized claim detail (UB-04 / CMS-1500)
- Relevant clinical documentation from the medical record